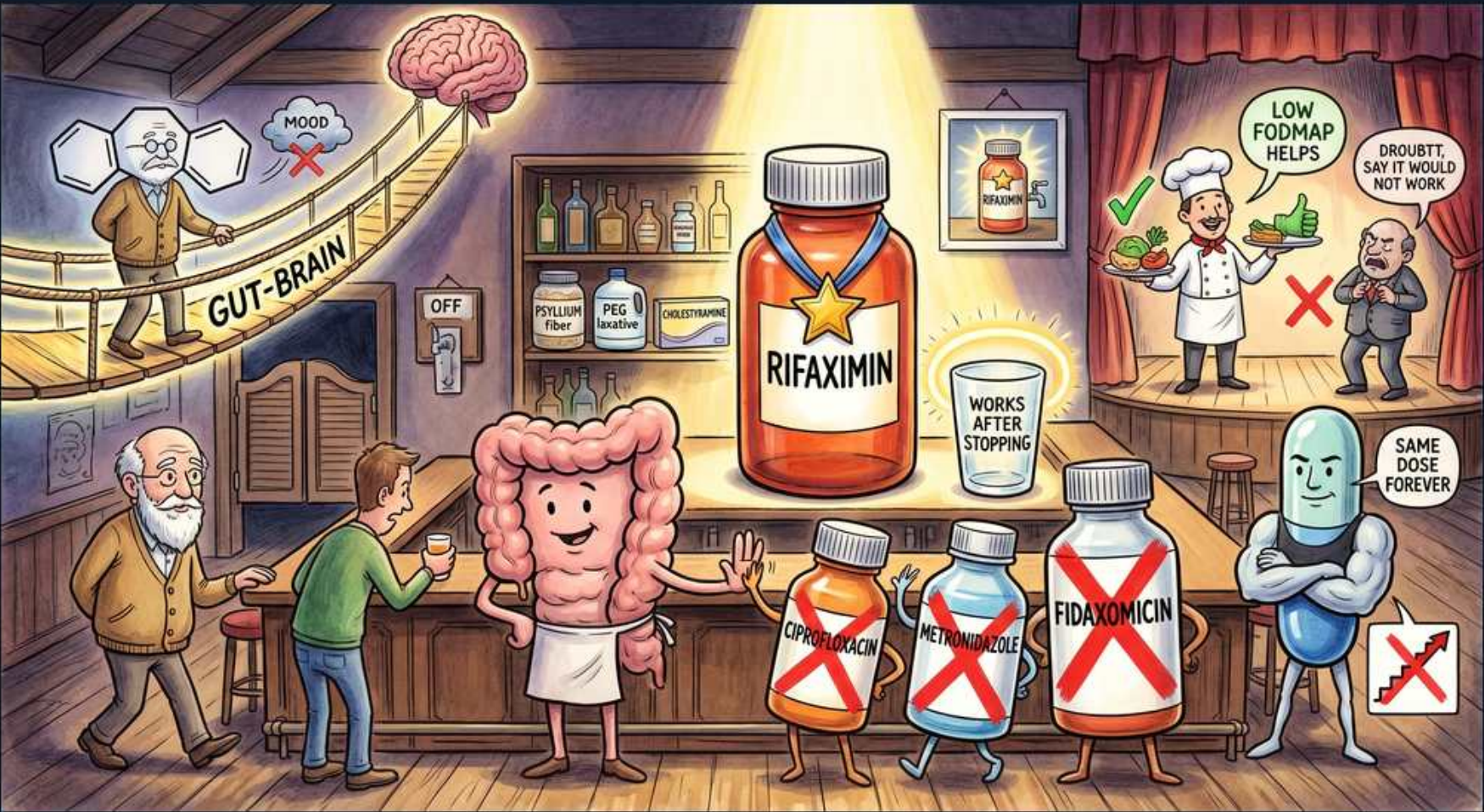


IBS — Treatment by Subtype



1. Rifaximin bottle (gold star)

WHAT YOU SEE

Large central bottle 'RIFAXIMIN' with a gold star/medal.

WHAT IT ENCODES

Rifaximin is the gold-standard antibiotic for IBS-D (non-constipation diarrhea-predominant). Non-absorbed (gut-luminal) so minimal systemic effect; targets small-bowel bacterial overgrowth. Typical course 550 mg TID x 14 days; it works again on retreatment after relapse.

2. Works after stopping glass

WHAT YOU SEE

A glass labeled 'WORKS AFTER STOPPING' beside the rifaximin bottle.

WHAT IT ENCODES

Rifaximin's benefit persists after a finite course, and a repeat course is effective if symptoms recur — no tolerance to its antibacterial effect. This durable, re-treatable response is a key tested feature.

3. Gut-brain catwalk bridge

WHAT YOU SEE

An overhead bridge/catwalk labeled 'GUT-BRAIN' with a brain above.

WHAT IT ENCODES

The gut-brain axis is the therapeutic target for neuromodulators. Low-dose TCAs (e.g., amitriptyline 10–25 mg) provide visceral analgesia and slow transit (helpful in IBS-D) through this axis — at doses far below antidepressant dosing.

4. Mood crossed out (red X)

WHAT YOU SEE

Brain/old man up top with 'MOOD' label crossed by a red X.

WHAT IT ENCODES

Low-dose TCAs in IBS work via gut-brain visceral analgesia, NOT via their antidepressant/mood effect. The analgesic mechanism is independent of and at lower dose than the mood effect — a classic board distinction.

BOARD TRAP

"TCAs help IBS through their antidepressant effect" → wrong; benefit is low-dose visceral analgesia via the gut-brain axis.

5. Shelf agents: psyllium / PEG / cholestyramine

WHAT YOU SEE

Back-bar shelf bottles labeled 'PSYLLIUM FIBER', 'PEG LAXATIVE', 'CHOLESTYRAMINE'.

WHAT IT ENCODES

Adjuncts: soluble psyllium fiber for global/IBS-C symptoms; PEG osmotic laxative for IBS-C constipation; cholestyramine (bile-acid sequestrant) for bile-acid diarrhea presenting as IBS-D. For IBS-C also linaclotide and lubiprostone (secretagogues).

6. Bartender gut

WHAT YOU SEE

Smiling intestine character behind the bar serving a customer.

WHAT IT ENCODES

The friendly gut anchors the IBS patient receiving subtype-tailored therapy. Treatment is matched to subtype: IBS-D (rifaximin, loperamide, cholestyramine, low-dose TCA) vs IBS-C (PEG, fiber, linaclotide/lubiprostone).

7. Low-FODMAP helps (green check)

WHAT YOU SEE

Chef holding a plate with a green check and sign 'LOW FODMAP HELPS'.

WHAT IT ENCODES

A low-FODMAP diet genuinely reduces IBS symptoms (bloating, pain, diarrhea) and is evidence-based first-line dietary therapy. Reintroduction phase identifies triggers; it is implemented with a dietitian to avoid nutritional gaps.

8. Doubter 'would not work' (red X)

WHAT YOU SEE

Skeptical man saying 'DOUBT IT, SAY IT WOULD NOT WORK', red X.

WHAT IT ENCODES

FALSE claim that low-FODMAP won't help. Evidence supports low-FODMAP as effective first-line dietary management of IBS — the doubter is wrong.

BOARD TRAP

"Low-FODMAP diet won't help IBS" → wrong; it is evidence-based and effective first-line dietary therapy.

9. Ciprofloxacin bottle (red X)

WHAT YOU SEE

Bottle 'CIPROFLOXACIN' with a red X.

WHAT IT ENCODES

Ciprofloxacin is NOT the IBS-D antibiotic — rifaximin is. Systemic fluoroquinolones carry tendon/QT/aortic and C. difficile risks and have no role in functional IBS.

BOARD TRAP

"Ciprofloxacin treats IBS-D" → wrong; rifaximin is the IBS-D antibiotic.

10. Metronidazole bottle (red X)

WHAT YOU SEE

Bottle 'METRONIDAZOLE' with a red X.

WHAT IT ENCODES

Metronidazole is not used for IBS-D; it targets anaerobes/protozoa (e.g., C. diff, Giardia). Rifaximin is the correct non-absorbed agent for IBS-D.

BOARD TRAP

"Metronidazole is the IBS-D antibiotic" → wrong; rifaximin is the one.

11. Fidaxomicin bottle (red X)

WHAT YOU SEE

Bottle 'FIDAXOMICIN' with a red X.

WHAT IT ENCODES

Fidaxomicin is for C. difficile infection, not IBS-D. Although also gut-selective, it is not an IBS therapy — rifaximin is.

BOARD TRAP

"Fidaxomicin treats IBS-D" → wrong; it is for C. difficile, not IBS; rifaximin is correct.

12. Loperamide 'same dose forever' capsule

WHAT YOU SEE

Blue capsule character labeled 'SAME DOSE FOREVER' with a stairs/red X below.

WHAT IT ENCODES

Loperamide controls IBS-D diarrhea with NO tolerance and NO dose escalation (the crossed-out staircase). It slows transit via gut μ -opioid receptors without central effect. Does not treat pain, so pair with a neuromodulator if pain dominant.

BOARD TRAP

"Loperamide requires progressive dose escalation" → wrong; it works at a stable dose with no tolerance.
